

RESPONSE TO HHS REQUEST FOR INFORMATION

"Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care"

Federal Register Docket No.: HHS Health Sector AI RFI

Submitted by:

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RE: AI Research Priorities for Medicare Fraud Prevention (Response to Question #10)

Dear HHS Deputy Secretary O'Neill and Assistant Secretary Dr. Keane:

I respectfully submit this response regarding specific areas of AI research that HHS should prioritize, with focus on fraud prevention—addressing HHS's stated goals of reducing healthcare costs and protecting taxpayer-funded programs.

BACKGROUND - THE \$100 BILLION PROBLEM:

Medicare fraud costs \$100+ billion annually. Current post-payment detection is reactive—CMS pays claims first, then investigative agencies spend years pursuing recovery. Today's DOJ announcement of Joel Rufus French's conviction for \$197M fraud (orthotic braces for amputees and deceased beneficiaries) demonstrates violations that AI could detect instantly.

RECOMMENDED PRIORITY: Real-Time Medicare Claims Validation Using AI

HHS should prioritize AI systems that prevent fraud BEFORE payment by validating claims in <3 seconds against regulatory and clinical standards.

TECHNICAL REQUIREMENTS:

- Regulatory compliance: AKS, Stark Law, False Claims Act, EKRA
- Clinical validation: ICD-10/CPT correlation, medical necessity, patient eligibility
- Pattern recognition: Statistical anomalies, emerging fraud schemes, financial relationships

AI/ML COMPONENTS:

- Supervised learning (trained on 20+ years OIG/DOJ prosecutions)
- Unsupervised learning (discovers novel patterns)
- NLP (analyzes documentation for gaming)
- Graph networks (maps ownership/kickback relationships)
- Reinforcement learning (optimizes based on outcomes)

PERFORMANCE TARGETS:

- Process: 15M+ claims daily

- Speed: <3 seconds per claim
- Accuracy: >95% detection rate
- Scale: 5B+ claims annually
- Uptime: 99.99%

VALUE PROPOSITION:

Current: Pay \$100B fraud, spend \$5B investigating, recover \$20B = Net loss \$85B

AI Prevention: Spend \$10B validation, prevent \$90B fraud = Net savings \$80B

ROI: 8:1 return

RECOMMENDED SBIR TOPICS FY2026-2027:

1. "AI-Powered Real-Time Medicare Claims Validation"

Phase I: Demonstrate <3 sec validation, >90% accuracy

Phase II: Process 100K claims/day prototype

Phase III: Scale to 1B claims/year nationally

2. "ML for Healthcare Billing Vulnerability Detection"

Phase I: Prove AI detects emerging fraud faster than analysts

Phase II: Real-time loophole detection system

Phase III: CMS integration for continuous monitoring

3. "NLP for Medical Necessity Validation"

Phase I: Detect documentation gaming >85% accuracy

Phase II: Production-grade medical record analysis

Phase III: Claims workflow integration

ALIGNMENT WITH HHS PRIORITIES:

- ✓ Reduces costs (\$80B annual savings)
- ✓ Reduces provider burden (guidance prevents violations)
- ✓ Protects patients (prevents unnecessary procedures)
- ✓ Maintains public trust (stewardship demonstration)
- ✓ American AI leadership (novel regulatory application)

COMMERCIAL APPLICABILITY:

- Government: Medicare/Medicaid
- Commercial: Private payers (\$60-80B fraud annually)
- Enables public-private partnerships

CONCLUSION:

AI fraud prevention shifts from reactive detection to proactive prevention. Technology is feasible, ROI compelling, need urgent. I welcome the opportunity to provide additional technical input.

Respectfully,

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